

Sugammadex: Drug information - UpToDate

Sugammadex: Drug information - UpToDate

Official reprint from UpToDate®

www.uptodate.com © 2026 UpToDate, Inc. and/or its affiliates. All Rights Reserved.

Sugammadex: Drug information

2026© UpToDate, Inc. and its affiliates and/or licensors. All Rights Reserved.

Contributor Disclosures

For additional information see "Sugammadex: Patient drug information" and "Sugammadex: Pediatric drug information"

For abbreviations, symbols, and age group definitions show table

Brand Names: US

- Bridion

Brand Names: Canada

- Bridion;
- PMS-Sugammadex

Pharmacologic Category

- Antidote;
- Selective Relaxant Binding Agent

Dosing: Adult

Note: Dosing based on actual body weight. Doses and timing of administration based on monitoring for twitch responses and the extent of spontaneous recovery that has occurred.

Routine reversal of rocuronium- or vecuronium-induced blockade

Routine reversal of rocuronium- or vecuronium-induced blockade:

IV:

Deep block (at least 1 to 2 post-tetanic counts and prior to the second twitch following train-of-four [TOF] stimulation): 4 mg/kg as a single dose (Ref).

Moderate block (after appearance of the second twitch following TOF stimulation): 2 mg/kg as a single dose.

Readministration of rocuronium or vecuronium: Following sugammadex use for routine reversal, waiting times for readministration of rocuronium or vecuronium vary greatly (5 minutes to 24 hours) depending on agent, dose, and renal function (consult product labeling); if immediate neuromuscular blockade is needed, a nonsteroidal neuromuscular-blocking agent (eg, cisatracurium or atracurium) may be required.

Immediate reversal of rocuronium-induced blockade

Immediate reversal of rocuronium-induced blockade: IV: 16 mg/kg as a single dose administered soon (~3 minutes) after administration of a single dose of rocuronium 1.2 mg/kg. **Note:** This dose of sugammadex has not been evaluated following administration of vecuronium.

Readministration of rocuronium or administration of vecuronium: Following sugammadex use for immediate reversal of rocuronium, wait 24 hours before readministering rocuronium or administering vecuronium. If more immediate neuromuscular blockade is needed, a nonsteroidal neuromuscular-blocking agent (eg, cisatracurium or atracurium) may be required.

Dosage adjustment for concomitant therapy: Significant drug interactions exist, requiring dose/frequency adjustment or avoidance. Consult drug interactions database for more information.

Dosing: Kidney Impairment: Adult

CrCl 30 to 80 mL/minute: No dosage adjustment necessary. After administration of up to 4 mg/kg of sugammadex and at least 24 hours has elapsed, a neuromuscular blocking agent (ie, rocuronium 0.6 mg/kg or vecuronium 0.1 mg/kg) may be readministered if necessary. If sooner readministration of a neuromuscular blocking agent is required, rocuronium 1.2 mg/kg may be administered (no dosing recommended for vecuronium).

CrCl < 30 mL/minute: Use is not recommended.

Dialysis: Use is not recommended.

Dosing: Liver Impairment: Adult

There are no dosage adjustments provided in the manufacturer's labeling (has not been studied); use with caution, particularly if accompanied by coagulopathy or severe edema.

Dosing: Obesity: Adult

The recommendations for dosing in patients with obesity are based upon the best available evidence and clinical expertise. Senior Editorial Team: Jeffrey F. Barletta, PharmD, FCCM; Manjunath P. Pai, PharmD, FCP; Jason A. Roberts, PhD, BPharm (Hons), B App Sc, FSHP, FISAC.

Class 3 obesity (BMI ≥ 40 kg/m²):

IV: 2 or 4 mg/kg (based on moderate or deep level of block) using **actual** or **adjusted** body weight (Ref).

Rationale for recommendation: Pharmacokinetic studies show sugammadex, a hydrophilic compound, has similar distribution and clearance whether dosed by **actual** or **ideal** body weight (Ref); however, dosing by **actual** body weight results in higher plasma concentrations and faster reversal times (Ref). Alternatively, dosing by **adjusted** body weight appears comparable to **actual** body weight in clinical reversal (Ref).

Dosing: Older Adult

Refer to adult dosing. Use with caution; reversal time may be delayed.

Dosing: Pediatric

(For additional information see "Sugammadex: Pediatric drug information")

Routine reversal of rocuronium- or vecuronium-induced blockade

Routine reversal of rocuronium- or vecuronium-induced blockade:

Note: Dosing based on actual body weight. Doses and timing of administration based on monitoring for twitch responses and the extent of spontaneous recovery that has occurred.

Infants, Children, and Adolescents:

Deep level of neuromuscular blockade (ie, spontaneous recovery of twitch response of 1 to 2 post-tetanic counts and zero twitch response to train-of-four [TOF] stimulation): IV bolus: 4 mg/kg as a single dose.

Moderate level of neuromuscular blockade (ie, after appearance of the second twitch in response to TOF stimulation): IV bolus: 2 mg/kg as a single dose.

Readministration of rocuronium or vecuronium: Following sugammadex use for routine reversal, waiting times for readministration of rocuronium or vecuronium vary greatly (5 minutes to 24 hours) depending on agent, dose, and kidney function (consult product labeling); if immediate neuromuscular blockade is needed, a nonsteroidal neuromuscular-blocking agent (eg, cisatracurium, atracurium) may be required.

Dosage adjustment for concomitant therapy: Significant drug interactions exist, requiring dose/frequency adjustment or avoidance. Consult drug interactions database for more information.

Dosing: Kidney Impairment: Pediatric

Infants, Children, and Adolescents:

CrCl 30 to 80 mL/minute/1.73 m²: IV: There are no pediatric-specific recommendations in manufacturer's labeling; based on experience in adult patients, no dosage adjustment necessary.

CrCl <30 mL/minute/1.73 m²: Use is not recommended (insufficient safety data combined with prolonged and increased overall drug exposure).

Dialysis: Use is not recommended.

Dosing: Liver Impairment: Pediatric

Infants, Children, and Adolescents: There are no dosage adjustments provided in the manufacturer's labeling (has not been studied); use with caution, particularly if accompanied by coagulopathy or severe edema.

Adverse Reactions

The following adverse drug reactions and incidences are derived from product labeling unless otherwise specified. Reported adverse reactions are for adults unless otherwise specified.

>10%:

Cardiovascular: Hypotension (5% to 13%)

Gastrointestinal: Nausea (adults: 26%; children, adolescents: 2% to 6%), vomiting (neonates, infants, children, adolescents, adults: 2% to 15%)

Nervous system: Pain (48% to 59%)

1% to 10%:

Cardiovascular: Bradycardia (children, adolescents, adults: 5% to 10%), ECG abnormality (6%; including prolonged QT interval on ECG), hypertension (9%), tachycardia (2% to 5%)

Dermatologic: Erythema of skin (2%), pruritus (3%)

Endocrine & metabolic: Hypocalcemia (2%)

Gastrointestinal: Abdominal pain (4% to 6%), flatulence (3%), xerostomia (≤2%)

Hematologic & oncologic: Decreased red blood cells (1% to 2%)

Hypersensitivity: Hypersensitivity reaction (7% to 9%; including severe hypersensitivity reaction; anaphylaxis [$<1\%$])

Nervous system: Anxiety (1% to 3%), chills (7%), depression (≤2%), dizziness (3% to 6%), headache (10%), hypoesthesia (3%), insomnia (5%), restlessness (1% to 2%)

Neuromuscular & skeletal: Increased creatine phosphokinase in blood specimen (1% to 2%), limb pain (6%), musculoskeletal pain (2%), myalgia (2%)

Ophthalmic: Ophthalmic signs and symptoms (children, adolescents: 1% to 6%)

Respiratory: Cough (3% to 8%)

Miscellaneous: Fever (neonates, infants, children, adults: 5% to 9%)

Frequency not defined: Hematologic & oncologic: Increased INR, prolonged partial thromboplastin time, prolonged prothrombin time

Postmarketing:

Cardiovascular: Atrial fibrillation, atrioventricular block, ST segment changes on ECG, supraventricular extrasystole, supraventricular tachycardia, ventricular fibrillation, ventricular tachycardia

Hypersensitivity: Anaphylactic shock, nonimmune anaphylaxis, type I hypersensitivity reaction

Neuromuscular & skeletal: Laryngospasm

Respiratory: Bronchospasm (Ruetzler 2022), dyspnea, pulmonary edema, wheezing

Contraindications

Hypersensitivity (eg, anaphylactic shock, anaphylaxis, skin reactions) to sugammadex or any component of the formulation.

Warnings/Precautions

Concerns related to adverse effects:

- Bradycardia: Marked bradycardia and bradycardia with cardiac arrest have been reported, usually within minutes after administration. Monitor closely for hemodynamic changes during and after reversal of neuromuscular blockade; use appropriate pharmacologic treatment (eg, atropine) if significant bradycardia occurs.
- Hypersensitivity: Serious hypersensitivity reactions (including anaphylaxis and anaphylactic shock) have been reported (uncommonly). May occur in patients without prior exposure to sugammadex.
- Recurrence of neuromuscular blockade: Recurrence of neuromuscular blockade has occurred in controlled trials, usually associated with suboptimal dosing. After initial reversal with sugammadex, continue respiratory monitoring and ensure adequate ventilator support remains accessible following extubation. Potentiation of neuromuscular blockade by other drugs used in the post-operative period should also be considered for the possibility of recurrence.

Disease-related concerns:

- Cardiovascular disease: Use caution in patients with cardiovascular disease.
- Impaired hemostasis: Use caution in patients with or at risk for impaired hemostasis (eg, coagulopathies, severe liver impairment, or concurrent use of anticoagulants at therapeutic doses). Dose-dependent transient increases in activated partial thromboplastin time (aPTT) and normalized prothrombin time (PT [INR]) have been observed. In clinical trials, significant effects on bleeding were not observed with low-dose sugammadex alone or in conjunction with therapeutic anticoagulation; high risk patients and those receiving high-dose sugammadex were not adequately studied. Careful monitoring of hemostatic and coagulation parameters is recommended.

Other warnings/precautions:

- Appropriate use: Use in intensive care (ICU) setting has not been evaluated. Do not use sugammadex for neuromuscular blockade induced by nonsteroidal neuromuscular blocking agents (eg, succinylcholine or benzylisoquinolinium compounds) or steroidal agents other than rocuronium or vecuronium.
- Experienced personnel: Must be administered under supervision of experienced health care provider familiar with its use.
- Light anesthesia: Signs of light anesthesia (eg, coughing, grimacing, movement, or suckling of tracheal tube) may become apparent when neuromuscular blockade is reversed intentionally in the middle of anesthesia.

- Respiratory function monitoring: Since other drugs (eg, opioids) used in the peri- and post-operative period may depress respiratory function, ventilatory support is mandatory until adequate spontaneous respiration is restored and ability to maintain a patent airway is assured. In addition, some patients experience a delayed or minimal response to recommended doses of sugammadex.

Dosage Forms: US

Excipient information presented when available (limited, particularly for generics); consult specific product labeling.

Solution, Intravenous [preservative free]:

Bridion: 200 mg/2 mL (2 mL); 500 mg/5 mL (5 mL)

Generic Equivalent Available: US

No

Pricing: US

Solution (Bridion Intravenous)

200 mg/2 mL (per mL): \$85.76

500 mg/5 mL (per mL): \$62.83

Disclaimer: A representative AWP (Average Wholesale Price) price or price range is provided as reference price only. A range is provided when more than one manufacturer's AWP price is available and uses the low and high price reported by the manufacturers to determine the range. The pricing data should be used for benchmarking purposes only, and as such should not be used alone to set or adjudicate any prices for reimbursement or purchasing functions or considered to be an exact price for a single product and/or manufacturer. Medi-Span expressly disclaims all warranties of any kind or nature, whether express or implied, and assumes no liability with respect to accuracy of price or price range data published in its solutions. In no event shall Medi-Span be liable for special, indirect, incidental, or consequential damages arising from use of price or price range data. Pricing data is updated monthly.

Dosage Forms: Canada

Excipient information presented when available (limited, particularly for generics); consult specific product labeling.

Solution, Intravenous:

Bridion: 100 mg/mL (2 mL, 5 mL)

Generic: 100 mg/mL (2 mL, 5 mL)

Administration: Adult

IV: Administer as rapid IV push over 10 seconds (according to the manufacturer); however, some experts suggest administering slow IV push to reduce incidence of serious adverse events (eg, bradycardia or asystole) (Ref). If administered in same IV line as other products, flush with NS before and after administration of sugammadex.

Administration: Pediatric

IV: Administer undiluted (100 mg/mL) or diluted (10 mg/mL) over 10 seconds; however, some experts suggest administering slow IV push to reduce the incidence of serious adverse events (eg, bradycardia, asystole) (Ref). If administered in same IV line as other products, adequately flush line (eg, with NS) before and after administration of sugammadex.

Preparation for Administration: Pediatric

IV: May be diluted in NS to a final concentration of 10 mg/mL; in trials, the dose has been diluted to a final concentration of 25 mg/mL (Ref).

Storage/Stability

Store intact vial at 25°C (77°F); excursions permitted to 15°C to 30°C (59°F to 86°F). Protect from light. When not protected from light, use intact vial within 5 days. Discard any unused portion of a punctured (in-use) vial immediately after use. Solutions diluted for infusion should be used immediately after preparation (**Note:** Prior to November 2022, the prescribing information stated the diluted solution could be stored for up to 48 hours refrigerated or at room temperature).

Use: Labeled Indications

Reversal of rocuronium or vecuronium: Reversal of neuromuscular blockade induced by rocuronium or vecuronium in adults and pediatric patients undergoing surgery.

Medication Safety Issues

Sound-alike/look-alike issues:

Sugammadex may be confused with sodium citrate.

Metabolism/Transport Effects

None known.

Drug Interactions

(For additional information: Launch drug interactions program)

Note: Interacting drugs may **not be individually listed below** if they are part of a group interaction (eg, individual drugs within “CYP3A4 Inducers [Strong]” are NOT listed). For a complete list of drug interactions by individual drug name and detailed management recommendations, use the drug interactions program by clicking on the “Launch drug interactions program” link above.

Anticoagulants: Sugammadex may increase anticoagulant effects of Anticoagulants. *Risk C: Monitor*

Fusidic Acid (Systemic): May decrease therapeutic effects of Sugammadex. *Risk C: Monitor*

Hormonal Contraceptives: Sugammadex may decrease therapeutic effects of Hormonal Contraceptives. Management: Patients receiving any hormonal contraceptive (oral or non-oral) should use an additional, nonhormonal contraceptive method during and for 7 days following sugammadex treatment. *Risk D: Consider Therapy Modification*

Toremifene: May decrease therapeutic effects of Sugammadex. *Risk C: Monitor*

Reproductive Considerations

The effects of hormonal contraception may be decreased following sugammadex administration. An additional nonhormonal contraceptive (eg, condom, spermicide) should be used for 7 days after a dose of sugammadex in women using oral or nonoral hormonal contraception.

Pregnancy Considerations

Adverse events have been observed in some animal reproduction studies. Limited information is available related to the use of sugammadex for the reversal of rocuronium-induced neuromuscular blockade after cesarean section (Pühringer 2010; Stourac 2013).

Breastfeeding Considerations

It is not known if sugammadex is excreted into breast milk. According to the manufacturer, the decision to continue or discontinue breast-feeding during therapy should take into account the risk of infant exposure, the benefits of breast-feeding to the infant, and the benefits of treatment to the mother.

Monitoring Parameters

Neuromuscular stimulation (eg, post-tetanic counts and train-of-four); ECG monitoring (for bradycardia) during and after administration (Hunter 2018); respiratory function during recovery; monitor for apnoea for at least 5 minutes after administration (Hotta 2016; Nakanishi 2016; Tsur 2014; Ue

2016); hemostatic and coagulation parameters in select patients (clinically significant bleeding has not been reported) (Rahe-Meyer 2014)

Mechanism of Action

Sugammadex is a modified gamma cyclodextrin which is a selective relaxant binding agent. It forms a complex with the neuromuscular-blocking agents rocuronium or vecuronium in plasma, reducing the amount of neuromuscular-blocking agent available to bind to nicotinic receptors in the neuromuscular junction. This results in the reversal of neuromuscular blockade induced by rocuronium or vecuronium.

Pharmacokinetics (Adult Data Unless Noted)

Onset: <3 minutes.

Distribution: V_d :

Neonates, Infants, and Children <2 years: ~1 to 3 L.

Children ≥2 and Adolescents <17 years: ~3 to 10 L.

Adults: 11 to 14 L.

Protein binding: Negligible.

Metabolism: Not metabolized.

Half-life elimination:

Neonates, Infants, Children, and Adolescents <17 years: ~1 to 2 hours.

Adult: Effective: ~2 hours; Prolonged in renal impairment: Mild renal impairment: 4 hours; Moderate renal impairment: 6 hours; Severe renal impairment: 19 hours.

Excretion: Urine (95% as unchanged drug).

Pharmacokinetics: Additional Considerations (Adult Data Unless Noted)

Pediatric: Clearance and volume of distribution increase with increasing age.

Patient Counseling Points

(For additional information see "Sugammadex: Patient drug information")

What is this drug used for?

- It is used to reverse the effects of some drugs.

All drugs may cause side effects. However, many people have no side effects or only have minor side effects. Call your doctor or get medical help if any of these side effects or any other side effects bother you or do not go away:

- Upset stomach or throwing up
- Stomach pain
- Dizziness or headache
- Cough
- Trouble sleeping
- Pain in arms or legs

WARNING/CAUTION: Even though it may be rare, some people may have very bad and sometimes deadly side effects when taking a drug. Tell your doctor or get medical help right away if you have any of the following signs or symptoms that may be related to a very bad side effect:

- Bleeding like throwing up or coughing up blood; vomit that looks like coffee grounds; blood in the urine; black, red, or tarry stools; bleeding from the gums; abnormal vaginal bleeding; bruises without a cause or that get bigger; or bleeding you cannot stop
- High or low blood pressure like very bad headache or dizziness, passing out, or change in eyesight
- Flushing
- Skin irritation
- Fast, slow, or abnormal heartbeat
- Chest pain
- Fever or chills
- Mouth or throat pain or irritation
- Signs of an allergic reaction, like rash; hives; itching; red, swollen, blistered, or peeling skin with or without fever; wheezing; tightness in the chest or throat; trouble breathing, swallowing, or talking; unusual hoarseness; or swelling of the mouth, face, lips, tongue, or throat.

Note: This is not a comprehensive list of all side effects. Talk to your doctor if you have questions.

Consumer Information Use and Disclaimer: This information should not be used to decide whether or not to take this medicine or any other medicine. Only the healthcare provider has the knowledge and training to decide which medicines are right for a specific patient. This information does not endorse any medicine as safe, effective, or approved for treating any patient or health condition. This is only a limited summary of general information about the medicine's uses from the patient education leaflet and is not intended to be comprehensive. This limited summary does NOT include all information available about the possible uses, directions, warnings, precautions, interactions, adverse effects, or risks that may apply to this medicine. This information is not intended to provide medical advice, diagnosis or treatment and does not replace information you receive from the healthcare provider. For a more detailed summary of information about the risks and benefits of using this medicine, please speak with your healthcare provider and review the entire patient education leaflet.

Brand Names: International

International Brand Names by Country

For country code abbreviations (show table)

- (AE) United Arab Emirates: Bridion;
- (AR) Argentina: Bridion;
- (AT) Austria: Bridion | Sugagelan | Sugammadex baxter | Sugammadex biosyn | Sugammadex fresenius kabi | Sugammadex gebro | Sugammadex hameln | Sugammadex juta | Sugammadex kalceks | Sugammadex mylan | Sugammadex ratiopharm | Sugammadex sandoz | Sugammadex sintetica | Sugammadex stada;
- (AU) Australia: Sugammadex accord | Sugammadex arx | Sugammadex dr reddys | Sugammadex junio | Sugammadex lupin | Sugammadex medsurge | Sugammadex sandoz | Sugammadex teva | Sugammadex viatris;
- (BE) Belgium: Sugammadex aspen | Sugammadex baxter | Sugammadex eg | Sugammadex fresenius kabi | Sugammadex gebro | Sugammadex mylan | Sugammadex reddy | Sugammadex sandoz;
- (BG) Bulgaria: Bridion | Sugammadex fresenius kabi | Sugammadex rompharm;
- (BR) Brazil: Bridion | Bryony | Dexperta | Lessav | Rebrive | Reverso | Sug | Sugammadex sodico | Sujma | Sunam | Suversadex;
- (CH) Switzerland: Bridion;
- (CL) Chile: Bridion | Moveradex | Suversadex;
- (CO) Colombia: Bridion;

- (CR) Costa Rica: Sugammadex;
- (CZ) Czech Republic: Bridion | Sugammadex fresenius kabi | Sugammadex reddy | Sugammadex sandoz | Sugammadex zentiva;
- (DE) Germany: Bridion | Sugammadex;
- (DO) Dominican Republic: Bridion;
- (EC) Ecuador: Bridion | Sugammadex;
- (EE) Estonia: Bridion | Sugammadex kalceks;
- (EG) Egypt: Bridion;
- (ES) Spain: Bridion | Sugammadex accord | Sugammadex aspen | Sugammadex baxter | Sugammadex bexal | Sugammadex cipla | Sugammadex dr reddys | Sugammadex fresenius kabi | Sugammadex glenmark | Sugammadex normon | Sugammadex orion | Sugammadex qilu | Sugammadex sala | Sugammadex sandoz | Sugammadex tarris | Sugammadex teva | Sugammadex vivanta;
- (FI) Finland: Sugammadex baxter | Sugammadex fresenius kabi | Sugammadex mylan | Sugammadex noridem | Sugammadex orion | Sugammadex qilu | Sugammadex reddy | Sugammadex sandoz | Sugammadex stada;
- (FR) France: Sugammadex fresenius kabi | Sugammadex kalceks | Sugammadex mylan | Sugammadex qilu | Sugammadex reddy pharma | Sugammadex reig jofre | Sugammadex teva | Sugammadex zentiva;
- (GB) United Kingdom: Sugammadex | Sugammadex sandoz;
- (GR) Greece: Bridion;
- (GT) Guatemala: Bridion;
- (HU) Hungary: Bridion | Sugammadex anfarm | Sugammadex gebro | Sugammadex onkogen | Sugammadex qilu | Sugammadex reddy | Sugammadex teva;
- (ID) Indonesia: Bridion;
- (IE) Ireland: Bridion | Sugammadex | Sugammadex aspen | Sugammadex clonmel | Sugammadex teva;
- (IN) India: Tromadex;
- (IQ) Iraq: Biomadex;
- (IT) Italy: Bridion | Sugammadex accord | Sugammadex aspen | Sugammadex eg | Sugammadex fisiopharma | Sugammadex fresenius kabi | Sugammadex medac | Sugammadex mylan | Sugammadex qilu | Sugammadex reddy | Sugammadex sandoz | Sugammadex sun | Sugammadex teva | Sugammadex tillomed | Sugammadex zentiva;
- (JO) Jordan: Bridion;
- (JP) Japan: Bridion | Sugammadex | Sugammadex f | Sugammadex maruishi | Sugammadex sandoz | Sugammadex towa;
- (KE) Kenya: Bridion;
- (KR) Korea, Republic of: Bريس | Bridion | Bristurn | Endion | Lovedion | Reverdion | Riverseon | Rocudion | Verodion;
- (KW) Kuwait: Bridion;
- (LB) Lebanon: Bridion;
- (LT) Lithuania: Bridion | Sugammadex fresenius kabi | Sugammadex kalceks;
- (LV) Latvia: Bridion;
- (MX) Mexico: Bridion | Parinex | Rebrive | Resudex;
- (MY) Malaysia: Bridion;
- (NL) Netherlands: Bridion | Sugammadex | Sugammadex accord | Sugammadex baxter | Sugammadex cf | Sugammadex gebro | Sugammadex hameln | Sugammadex hikma | Sugammadex kalceks | Sugammadex mylan | Sugammadex sandoz | Sugammadex teva | Sugammadex vivanta;
- (NO) Norway: Bridion | Sugammadex baxter | Sugammadex bioglan | Sugammadex mylan | Sugammadex teva;
- (NZ) New Zealand: Sugammadex bnm | Sugammadex pharmazac;
- (PA) Panama: Bridion;
- (PE) Peru: Bridion | Dexperta | Rebrive;
- (PH) Philippines: Bridion;
- (PL) Poland: Bridion | Sugammadex aspen | Sugammadex delfarma | Sugammadex fresenius

- kabi | Sugammadex ranbaxy | Sugammadex reddy | Sugammadex reig jofre | Sugammadex sandoz | Sugammadex stada | Sugammadex teva | Sugammadex zentiva;
- (PR) Puerto Rico: Bridion;
 - (PT) Portugal: Bridion | Sugamadex mylan | Sugamadex sandoz | Sugamadex teva;
 - (QA) Qatar: Bridion | Plibadex | Sumadix;
 - (RO) Romania: Bridion | Sugammadex kalceks | Sugammadex pharmazac | Sugammadex reddy | Sugammadex rompharm | Sugammadex sandoz | Sugammadex terapia | Sugammadex teva;
 - (RU) Russian Federation: Bridion;
 - (SA) Saudi Arabia: Bridion | Surlex;
 - (SE) Sweden: Bridion | Sugammadex 1a farma | Sugammadex baxter | Sugammadex bioglan | Sugammadex fresenius kabi | Sugammadex gebro | Sugammadex glenmark | Sugammadex kalceks | Sugammadex mylan | Sugammadex noridem | Sugammadex orion | Sugammadex qilu | Sugammadex reddy | Sugammadex teva | Sugammadex zentiva;
 - (SG) Singapore: Bridion;
 - (SI) Slovenia: Sugamadeks amomed;
 - (SK) Slovakia: Bridion | Sugammadex amomed | Sugammadex fresenius kabi | Sugammadex kalceks | Sugammadex reddy;
 - (TH) Thailand: Bridion;
 - (TN) Tunisia: Bridion;
 - (TR) Turkey: Bridion | Brimadeks | Sugaver | Sugawake | Sugrino;
 - (UA) Ukraine: Sugammadex vista;
 - (UY) Uruguay: Reverso | Sugammadex;
 - (VE) Venezuela, Bolivarian Republic of: Bridimed;
 - (ZA) South Africa: Brevistell iv | Bridion | Desugomyl | Myladion | Senuom | Sugammadex sandoz | Sumabid | Temadex

Use of UpToDate is subject to the Terms of Use.

REFERENCES

1. Bridion (sugammadex) injection [prescribing information]. Rahway, NJ: Merck Sharp & Dohme LLC; March 2026.
2. Bridion solution for injection (sugammadex sodium) [product monograph]. Hertfordshire, UK; Merck Sharp & Dohme Limited; 2015.
3. Bridion solution for injection (sugammadex sodium) [summary of product characteristics]. Haarlem, Netherlands; Merck Sharp & Dohme B.V.; June 2013.
4. Donati F. Sugammadex: a cyclodextrin to reverse neuromuscular blockade in anaesthesia. *Expert Opin Pharmacother*. 2008;9(8):1375-1386. [PubMed 18473711]
5. Elfawy DM, Saleh M, Nofal WH. Sugammadex based on ideal, actual, or adjusted body weights for the reversal of neuromuscular blockade in patients undergoing laparoscopic bariatric surgery. *Research and Opinion in Anesthesia and Intensive Care*. 2019;6(1):20-26. doi:10.4103/roaic.roaic_12_18
6. Geldner G, Niskanen M, Laurila P, et al. A randomised controlled trial comparing sugammadex and neostigmine at different depths of neuromuscular blockade in patients undergoing laparoscopic surgery. *Anaesthesia*. 2012;67(9):991-998. doi:10.1111/j.1365-2044.2012.07197.x [PubMed 22698066]
7. Horrow JC, Li W, Blobner M, et al. Actual versus ideal body weight dosing of sugammadex in morbidly obese patients offers faster reversal of rocuronium- or vecuronium-induced deep or moderate neuromuscular block: a randomized clinical trial. *BMC Anesthesiol*. 2021;21(1):62. doi:10.1186/s12871-021-01278-w [PubMed 33639839]
8. Hotta E, Tamagawa-Mineoka R, Masuda K, et al. Anaphylaxis caused by γ -cyclodextrin in sugammadex. *Allergol Int*. 2016;65(3):356-358. doi:10.1016/j.alit.2016.02.013. [PubMed 27062217]

9. Hunter JM, Naguib M. Sugammadex-induced bradycardia and asystole: how great is the risk? *Br J Anaesth.* 2018;121(1):8-12. doi:10.1016/j.bja.2018.03.003. [PubMed 29935599]
10. Li D, Wang Y, Zhou Y, Yin C. Efficacy and safety of sugammadex doses calculated on the basis of corrected body weight and total body weight for the reversal of deep neuromuscular blockade in morbidly obese patients. *J Int Med Res.* 2021;49(1):300060520985679. doi:10.1177/0300060520985679 [PubMed 33499679]
11. Monk TG, Rietbergen H, Woo T, Fennema H. Use of sugammadex in patients with obesity: a pooled analysis. *Am J Ther.* 2017;24(5):e507-e516. doi:10.1097/MJT.0000000000000305 [PubMed 26398716]
12. Mostoller K, Wrishko R, Maganti L, Herring WJ, van Zutphen-van Geffen M. Pharmacokinetics of sugammadex dosed by actual and ideal body weight in patients with morbid obesity undergoing surgery. *Clin Transl Sci.* 2021;14(2):737-744. doi:10.1111/cts.12941 [PubMed 33278332]
13. Nakanishi T, Ishida K, Utada K, et al. Anaphylaxis to sugammadex diagnosed by skin prick testing using both sugammadex and a sugammadex-rocuronium mixture. *Anaesth Intensive Care.* 2016;44(1):122-124. [PubMed 26673602]
14. Plaud B, Meretoja O, Hofmockel R, et al. Reversal of rocuronium-induced neuromuscular blockade with sugammadex in pediatric and adult surgical patients. *Anesthesiology.* 2009;110:284-294. [PubMed 19194156]
15. Pühringer FK, Kristen P, Rex C. Sugammadex reversal of rocuronium-induced neuromuscular block in Caesarean section patients: a series of seven cases. *Br J Anaesth.* 2010;105(5):657-660. [PubMed 20736231]
16. Rahe-Meyer N, Fennema H, Schulman S, et al. Effect of reversal of neuromuscular blockade with sugammadex versus usual care on bleeding risk in a randomized study of surgical patients. *Anesthesiology.* 2014;121(5):969-977. doi:10.1097/ALN.0000000000000424 [PubMed 25208233]
17. Refer to manufacturer's labeling.
18. Ruetzler K, Li K, Chhabada S, et al. Sugammadex versus neostigmine for reversal of residual neuromuscular blocks after surgery: a retrospective cohort analysis of postoperative side effects. *Anesth Analg.* 2022;134(5):1043-1053. doi:10.1213/ANE.00000000000005842 [PubMed 35020636]
19. Stourac P, Krikava I, Seidlova J, et al. Sugammadex in a parturient with myotonic dystrophy. *Br J Anaesth.* 2013;110(4):657-658. [PubMed 23508494]
20. Subramani Y, Rajarathinam M, Dabirzadeh A, et al. Comparison of different weight scalars to dose sugammadex for the reversal of neuromuscular blockade in morbidly obese patients: a systematic review. *Cureus.* 2024;16(3):e57057. doi:10.7759/cureus.57057 [PubMed 38681306]
21. Tsur A, Kalansky A. Hypersensitivity associated with sugammadex administration: a systematic review. *Anaesthesia.* 2014;69(11):1251-1257. doi:10.1111/anae.12736. [PubMed 24848211]
22. Ue KL, Kasternow B, Wagner A, et al. Sugammadex: An emerging trigger of intraoperative anaphylaxis. *Ann Allergy Asthma Immunol.* 2016;117(6):714-716. doi:10.1016/j.anai.2016.09.428. [PubMed 27789121]

Topic 105866 Version 139.0

Source: UpToDate. Downloaded 2026-05-26.